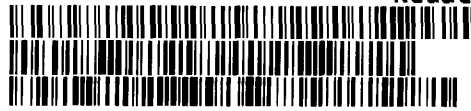


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TCGA-K4-A4AC-01A-PR

Redacted



Name

Encounter Number

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## SURGICAL PATHOLOGY

Time Collected  
Time Reported  
Ordering Provider  
Status

Time Received  
Order Number

Results

Source of Specimen

- A. LEFT DISTAL URETER-FS-
- B. RIGHT DISTAL URETER-FS-
- C. BLADDER, PROSTATE AND SEMINAL VESICLES-
- D. LEFT PELVIC LYMPH NODES-
- E. RIGHT PELVIC LYMPH NODES-

1CD-0-3

carcinoma, urothelial, NOS 8120/3

Site: bladder, NOS C67.9

FINAL DIAGNOSIS:-

- A. LEFT DISTAL URETER-FS-  
NO CARCINOMA SEEN.
- B. RIGHT DISTAL URETER-FS-  
NO CARCINOMA SEEN.
- C. BLADDER, PROSTATE AND SEMINAL VESICLES-  
HIGH GRADE UROTHELIAL CARCINOMA AND HIGH GRADE PROSTATIC  
ADENOCARCINOMA, SEE SYNOPTIC REPORTS BELOW.

fw  
9/24/12

BLADDER :  
INVASIVE CARCINOMA, HIGH GRADE.

TUMOR SITE: URINARY BLADDER.

HISTOLOGIC TYPE: UROTHELIAL.

SQUAMOUS DIFFERENTIATION: PRESENT.

GLANDULAR DIFFERENTIATION: NOT PRESENT.

TUMOR SIZE: 6 CM.

Prep

HISTOLOGIC GRADE: 3/3.

DEPTH OF INVASION: OUTER MUSCULARIS PROPRIA.

SPECIMEN TYPE: RADICAL CYSTOPROSTATECTOMY

TNM STAGE: pT2b, pN0, pMX.

LYMPH NODE(S) INCLUDED IN ALL PARTS: NUMBER INVOLVED : 0  
NUMBER EXAMINED: 5

LYMPHATIC (SMALL VESSEL) INVASION: NOT SEEN.

ASSOCIATED EPITHELIAL LESIONS: UROTHELIAL CARCINOMA IN SITU  
IDENTIFIED.

NO UROTHELIAL CARCINOMA SEEN AT MARGINS.

PROSTATE :  
ADENOCARCINOMA OF PROSTATE WITH INVASION OF BILATERAL SEMINAL  
VESICLES.

EXTRAPROSTATIC SOFT TISSUE: INVOLVED.

TUMOR SITE: BILATERAL AND DIFFUSE.

SPECIMEN TYPE: RADICAL CYSTOPROSTATECTOMY.

SIZE: 9 x 6 x 4.5 CM.

GLEASON SCORE (PRIMARY + SECONDARY PATTERN): 5 + 4 = 9/10.

TERTIARY PATTERN: PRESENT, GLEASON GRADE 3/5.

SEVERITY IN EXTENT OF TUMOR: 5 /5.

DOMINANT NODULE (AT LEAST 1 CM IN SIZE): NOT PRESENT.

TREATMENT EFFECT ON CARCINOMA: NOT IDENTIFIED.

TNM STAGE: pT3b, pN0, pMX.

LYMPHATIC (SMALL VESSEL) INVASION: SEEN.

PERINEURAL INVASION: PRESENT.

MARGIN STATUS (PROSTATE ADENOCARCINOMA):  
POSITIVE INKED MARGINS: LEFT PROSTATIC BASE AND RIGHT APEX,  
TRANSECTED.

Prepa

OTHER MARGINS ARE CLEAR.

D. LEFT PELVIC LYMPH NODES-

NO CARCINOMA SEEN IN THREE LYMPH NODES (0/3).

E. RIGHT PELVIC LYMPH NODES-

NO CARCINOMA SEEN IN TWO LYMPH NODES (0/2).

Diagnosis Note

C. HIGH GRADE UROTHELIAL CARCINOMA INVOLVES PROSTATIC PERIURETHRAL DUCTS BUT NO INVASIVE UROTHELIAL CARCINOMA IS SEEN IN THE PROSTATE GLAND.

THE PROSTATIC ADENOCARCINOMA SHOWS AREAS OF SIGNET RING MORPHOLOGY.

PAS IMMUNOSTAIN WAS EXAMINED TO DIFFERENTIATE INVASIVE PROSTATIC CARCINOMA FROM POSSIBLE INVASIVE UROTHELIAL CARCINOMA IN BLOCK C16 AND THE INVASIVE TUMOR WAS POSITIVE WHILE IN SITU UROTHELIAL CARCINOMA PRESENT IN THE SECTION WAS NEGATIVE.

Appropriate positive controls were stained along side sections from this case. The control tissue was examined and judged to be technically acceptable.

Signed Others

Frozen Section

A. LEFT DISTAL URETER: NO TUMOR SEEN.

RIGHT DISTAL URETER: NO TUMOR SEEN.

Case Clinical Information

BLADDER CANCER

Gross Description

A. Received in formalin labeled with the patient's name and "left distal ureter-FS" brown-tan soft tissue measuring 6 x 4 x 2 mm thick. The specimen is totally submitted in A1.

B. Received in formalin labeled with the patient's name and

Prep.

"right distal ureter FS" is a yellow-tan soft tissue measuring 5 x 3 x 2 mm. The specimen is submitted totally in B1.

C. Received in formalin labeled with the patient's name and "bladder, prostate and seminal vesicles" is a specimen so designated measuring overall 16 cm superior to inferior; 11 cm right to left; 7.5 cm anterior to posterior. The prostate itself seems enlarged measuring 9 cm anterior to posterior, 6 cm right to left, 4.5 cm superior to inferior. The bladder cavity measures 5.5 cm superior to inferior, 3.5 cm right to left, 3 cm anterior to posterior, and is completely filled with friable pale tan tumor apparently broadly attached posteriorly associated with pronounced induration of the bladder wall. On examination of the posterior aspect the specimen segments of right and left vas extend for 4 cm on each side. The area of the right and left seminal vesicles is rather indurated with fat surrounding the seminal vesicles somewhat more firm and pale tan than usual. Scattered staple lines are present along the posterior resection margin. The right side of the specimen is inked blue, the left side black. Upon sectioning, there appears to be tumor extending from the posterior prostate into the area of the right and left seminal vesicles bilaterally abutting the resection margin with the left seminal vesicle practically obliterated and the right better preserved. The tumor appears to abut the inked resection margin both in the area of the right and left seminal vesicle. Section code: C1=shaved left ureter; C2=shaved right ureter; C3-C4=distal prostatic urethra shaved from remainder of prostate and sectioned perpendicular to inked margin; C5-C9=extensive sampling tumor surrounding left seminal vesicle with inked surgical margin abutted; C10-C15=extensive sampling right seminal vesicle including tumor and focally abutted resection margin; C16=junction left posterior prostate and periprostatic extension; C17=sagittal plane section posterior mid bladder with invasive tumor measuring up to 2 cm thick; C18=left lateral bladder wall anterior; C19=right lateral bladder wall anterior superior; C20-C21=right lateral bladder wall full thickness section extending from papillary component of tumor to serosal aspect. The tumor overall measures 6 x 4 x 4 cm with entire bladder wall involved except limited area at base.

D. Received in formalin labeled with the patient's name and "left pelvic lymph nodes" are multiple fragments of fibrofatty tissue measuring 3 x 3 x 3 cm in compact aggregate. A dominant area of indurated fat bisected and totally submitted in D1-D2 and two additional areas of induration totally submitted split in D3-D4.

E. Received in formalin labeled with the patient's name and "right pelvic lymph nodes" are multiple fragments of fibroadipose tissue measuring 2.5 x 2.5 x 2.5 cm in compact aggregate. Dominant induration split totally submitted in E1-E2

and an additional area of induration totally submitted in E3.

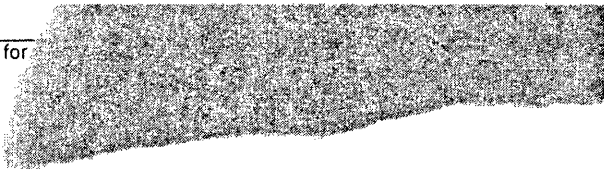
Procedure

A. AA ROUTINE H&E X1 BLOCK  
H&E X1 A RECUTS  
B. AA ROUTINE H&E X1 BLOCK  
H&E X1 A RECUTS  
C. AA ROUTINE H&E X1 BLOCK.1  
H&E X1  
C. AA ROUTINE H&E X1 BLOCK.2  
H&E X1  
C. AA ROUTINE H&E X1 BLOCK.3  
H&E X1 A RECUTS  
C. AA ROUTINE H&E X1 BLOCK.4  
H&E X1 A RECUTS  
C. AA ROUTINE H&E X1 BLOCK.5  
H&E X1  
C. AA ROUTINE H&E X1 BLOCK.6  
H&E X1  
C. AA ROUTINE H&E X1 BLOCK.7  
H&E X1  
C. AA ROUTINE H&E X1 BLOCK.8  
H&E X1  
C. AA ROUTINE H&E X1 BLOCK.9  
H&E X1  
C. AA ROUTINE H&E X1 BLOCK.10  
H&E X1  
C. AA ROUTINE H&E X1 BLOCK.11  
H&E X1  
C. AA ROUTINE H&E X1 BLOCK.12  
H&E X1  
C. AA ROUTINE H&E X1 BLOCK.13  
H&E X1  
C. AA ROUTINE H&E X1 BLOCK.14  
H&E X1  
C. AA ROUTINE H&E X1 BLOCK.15  
H&E X1  
C. AA ROUTINE H&E X1 BLOCK.16  
H&E X1 PSA  
C. AA ROUTINE H&E X1 BLOCK.17  
H&E X1  
C. AA ROUTINE H&E X1 BLOCK.18  
H&E X1  
C. AA ROUTINE H&E X1 BLOCK.19

Prepared for

H&E X1  
 C. AA ROUTINE H&E X1 BLOCK.20  
 H&E X1  
 C. AA ROUTINE H&E X1 BLOCK.21  
 H&E X1  
 D. AA ROUTINE H&E X1 BLOCK.1  
 H&E X1  
 D. AA ROUTINE H&E X1 BLOCK.2  
 H&E X1  
 D. AA ROUTINE H&E X1 BLOCK.3  
 H&E X1  
 D. AA ROUTINE H&E X1 BLOCK.4  
 H&E X1  
 E. AA ROUTINE H&E X1 BLOCK.1  
 H&E X1  
 E. AA ROUTINE H&E X1 BLOCK.2  
 H&E X1  
 E. AA ROUTINE H&E X1 BLOCK.3  
 H&E X1

Prepared for



| Criteria                        | Yes                                                | No |
|---------------------------------|----------------------------------------------------|----|
| Diagnosis Discrepancy           |                                                    |    |
| Primary Tumor Site Discrepancy  |                                                    |    |
| IPAA Discrepancy                |                                                    |    |
| Prior Malignancy History        |                                                    |    |
| Qual. (continued) Primary Noted |                                                    |    |
| Case is (continued)             |                                                    |    |
| Reviewer Initials               | QUALIFIED / DISQUALIFIED<br>Date Reviewed: 9/11/12 |    |

9/11/12